

CPT 2027 Maternity Care Transition

RESEARCH VALIDATED — AMA Coding Primer Webinar, June 2, 2026

All guidance in this playbook is validated against the AMA's official CPT 2027 Maternity Care Services Code Changes documentation and the AMA's free public coding primer webinar held June 2, 2026 at 12:00 PM CDT — titled 'A Coding Primer: Previewing the CPT 2027 Restructure for Maternity Care Services Codes.' The webinar confirmed the four-phase framework, new code structure, and January 1, 2027 effective date. CMS will publish final RVU values in November 2026.

EXECUTIVE OVERVIEW

Effective **January 1, 2027**, the American Medical Association (AMA) will eliminate all bundled global obstetric CPT codes — a change 30+ years in the making. In their place, maternity care will be reported across **four distinct phases**: Antepartum, Labor Management, Delivery, and Postpartum. This playbook provides our OB/GYN department with a structured, step-by-step roadmap to educate providers and staff, redesign billing workflows, update EHR templates, and coordinate with HP on payer contracts — all before the January 1, 2027 go-live.

35

Total CPT Codes
Changed

17

Codes Deleted

12

New Codes Added

6

Codes Revised

4

Phases of Care

SECTION 1 | BACKGROUND & WHAT IS CHANGING

What Is the Global OB Package?

Since the early 1990s, maternity care in the United States has been billed under a '**global package**' model — a single bundled CPT code representing the full continuum of a patient's prenatal, delivery, and postpartum care as one reimbursable event. This model simplified billing but masked the complexity, volume, and variety of care that modern OB/GYN teams deliver.

Why Is It Changing?

Following nearly two years of collaborative work between the AMA CPT Editorial Panel, ACOG, and national specialty societies — and confirmed at the AMA's Coding Primer Webinar on **June 2, 2026** — the global package is being dismantled because:

- Pregnancy care is now routinely delivered by multiple, sometimes unaffiliated care teams across antepartum, labor, delivery, and postpartum phases.
- Patients are frequently transferred from rural hospitals to facilities equipped for high-risk care — scenarios the bundled code never adequately captured.
- The global model obscured care variation, complexity, and cost transparency.
- Increasing use of home monitoring, telehealth, and additional postpartum monitoring (mental health, hemorrhage, cardiac conditions — the leading causes of maternal death) requires more granular reporting.
- Maternal-Fetal Medicine (MFM) and high-risk practices were systematically underpaid relative to the volume and complexity of services rendered.

AMA Research Validation — Recent Conference Confirmation

AMA CODING PRIMER WEBINAR — JUNE 2, 2026

The AMA hosted a free public webinar on **June 2, 2026** specifically confirming all CPT 2027 maternity care restructuring details. Topics covered: practical coding guidance for all four phases (antepartum, labor management, delivery, postpartum); E/M coding requirements; and the January 1, 2027 effective date. A companion **Health Plan Primer** was simultaneously released for payers. The AMA also confirmed that **CMS will propose RVU values in July 2026**, with final values published in **November 2026**. All guidance in this playbook aligns with both the AMA's official CPT documentation and the June 2026 coding primer.

Scope of Code Changes (Effective January 1, 2027)

Action	Count	Examples / Notes
Codes Deleted	17	59400, 59409, 59410, 59425, 59426, 59430, 59510, 59515, 59610, 59618, and others
New Codes Added	12	New labor management codes (initial/subsequent, straightforward/complex); new delivery-only codes; laceration repair codes; uterine tamponade code

Codes Revised	6	Delivery and postpartum codes updated to stand-alone (non-global) reporting
Total Changes	35	Largest restructuring of obstetric coding in 30+ years

The Four New Phases of Maternity Care Reporting

PHASE 1 — ANTEPARTUM

Every prenatal visit billed individually using E/M codes (CPT 99202–99499). HCPCS modifier **TH** appended to differentiate from non-obstetric visits. Code selection based on Medical Decision-Making (MDM) or total time per encounter. Current antepartum codes 59425, 59426 and all global codes are deleted. Ultrasound and fetal MRI continue to be billed separately.

PHASE 2 — LABOR MANAGEMENT

Reported daily (once per calendar date). Distinguishes **initial vs. subsequent** days and **straightforward vs. complex** management. Includes interim physical exams, collection/interpretation of physiological data, and induction/augmentation of labor. Labor management is billed separately from delivery.

PHASE 3 — DELIVERY

Streamlined stand-alone codes: vaginal delivery, vaginal delivery + episiotomy, VBAC, primary cesarean, repeat cesarean. Delivery codes do NOT include labor management or postpartum care. Routine same-day postpartum care is incorporated in the delivery code. New separate codes for 3rd- and 4th-degree laceration repair.

PHASE 4 — POSTPARTUM

All current postpartum codes deleted. Reported per encounter with E/M codes + modifier TH. Inpatient post-delivery days billed with subsequent hospital care codes. Discharge billed with discharge management code. Outpatient postpartum visits billed with office E/M + modifier TH.

SECTION 2 | IMPACT ON OUR ORGANIZATION

Revenue & Volume Impact

Under the old global model, a typical pregnancy generated **1 claim** for the full episode. Under the new unbundled model, that same pregnancy will generate an estimated **15× more claims** — one per prenatal visit (avg. 10–14), multiple labor management days, a delivery claim, and multiple postpartum visits. This is a fundamental operational shift.

Area	Old Model	New Model (Jan 1, 2027)
Claims per pregnancy	1 global claim	~15+ individual claims
Antepartum billing	Bundled into global code	E/M code per visit + modifier TH
Labor management	Included in global code	Separate daily codes (initial/subsequent; straightforward/complex)
Delivery	Bundled	Stand-alone delivery code only
Postpartum	Bundled into global code	E/M code per visit + modifier TH
Documentation burden	Moderate	HIGH — every visit must independently justify E/M level
Payer contracting	Single global rate	Requires HP-led renegotiation of per-code rates
EHR templates	Minimal change needed	Must be updated for E/M documentation + all new codes
Revenue potential	Fixed global rate	Higher for complex/high-risk; risk if under-documented

Risks If We Are Not Prepared

- **Claim denials** — payers without renegotiated contracts will auto-deny new codes.
- **Revenue gap** — practices billing deleted global codes after 1/1/2027 receive no payment.
- **Audit exposure** — under-documented E/M levels invite post-payment audits.
- **Patient confusion** — patients receiving multiple EOBs instead of one may call billing.
- **Staff overwhelm** — a 15× increase in claim volume without workflow changes breaks the billing cycle.

SECTION 3 | MASTER IMPLEMENTATION TIMELINE

All milestones are sequenced to ensure our OB/GYN providers are fully trained, all three education modules are deployed, physician EHR templates are updated, and our intake/communication system is live well ahead of the January 1, 2027 go-live.

Phase	Milestone	Target Date	Owner
NOW (June 2026)	Project Kickoff — Assemble core project team (super users, HP lead, IT/EHR lead, practice manager, department medical director). Review AMA June 2 webinar recording. Assign module ownership.	June 2026	Dept. Lead
	Needs Assessment — Audit current global code usage (59400, 59510, etc.). Baseline current claim volume. Identify payers for HP-led contract renegotiation. Assess EHR documentation gaps.	June 2026	HP
PHASE 1 OFFICE VISITS	Module 1 Video — First Draft (Office Visits) — Create video (less than 15 minutes) covering antepartum/postpartum E/M code selection (99202–99499), modifier TH, MDM documentation, time-based coding, and ICD-10-CM pregnancy codes.	July 1, 2026	Education Team
	Update Physician EHR Templates — Office Visits — Revise antepartum and postpartum office visit note templates to include MDM documentation elements and time capture fields required for E/M code selection. Coordinate with IT/EHR team and review with super users.	July 2026	IT / EHR Team
	Super User Review & Pilot — Module 1 — Super users review video draft, validate accuracy against AMA guidelines, pilot coding on 10 real antepartum visit notes. Document any corrections.	July 2026	Super Users
	Module 1 Finalized & Published — Incorporate feedback. Publish Module 1 (Office Visits) + quick-reference card to SharePoint. Track completion.	August 2026	Education + IT

PHASE 2 INPATIENT	Module 2 Video — Inpatient — Develop video (less than 15 minutes) covering labor management codes (initial/subsequent, straightforward/complex), delivery-only codes, subsequent hospital care, and discharge coding.	July–Aug 2026	Education Team
	Update Physician EHR Templates — Inpatient / Delivery — Revise labor management documentation templates, delivery note templates, and subsequent hospital care note templates. Separate labor management documentation from delivery documentation. Validate in EHR sandbox with IT team.	August 2026	IT / EHR Team
	Super User Review & Pilot — Module 2 — Validate inpatient module against AMA guidelines. Run tabletop coding scenarios. Pilot with hospitalist OB team.	August 2026	Super Users
	Inpatient Module Published — Publish Module 2 + scenario workbook to SharePoint.	August 2026	Education + IT
PHASE 3 OB CO MPREHENSIVE	Module 3 Video — OB Comprehensive — Build video (less than 15 minutes) as the capstone: full episode from confirmation through postpartum, edge cases (VBAC, C-section, split care/transfer), audit risk scenarios.	August 2026	Education Team
	Super User Review & Pilot — Module 3 — Full scenario testing. Build FAQ bank. Co-develop live Q&A; session content.	August 2026	Super Users
	All Three Modules Live in SharePoint — Full module library, attestation quizzes, and quick-reference cards published.	Late Aug 2026	Education + IT

PHASE 4 PROVIDER TRAINING	All OB/GYN Providers Trained — September 1 Target — All physicians complete all three modules. Live Q&A; sessions held. Attestation recorded. Intake system live for SharePoint-routed questions.	Sept 1, 2026	Dept. Director
	HP Billing Staff & Front Desk Training — HP-led training on new claim submission workflow, modifier TH, prior auth changes, and patient EOB communication. Coordinate with HP leadership on staff readiness.	Sept 2026	HP
	Payer Contract Renegotiations — HP Led — HP coordinates with HP leadership to renegotiate all major payer contracts for new per-code rates. Confirm no global code carve-outs remain. Written confirmations requested.	Sept–Oct 2026	HP
	EHR / EMR Go-Live Readiness — All new codes validated in sandbox. Old global codes removed. New labor management and delivery codes active. Utilization monitoring report built.	Oct–Nov 2026	IT / EHR Team
GO-LIVE JAN 1, 2027	January 1, 2027 — New CPT Codes Effective — All global OB codes officially deleted. All claims under new code structure. SharePoint dashboard active for ongoing questions and HP communication.	Jan 1, 2027	All Teams
	30-Day Post-Go-Live Audit — HP reviews claim acceptance/denial rates. Documentation quality review. Super users provide live support. Rapid corrections issued.	Feb 2027	HP
	60/90-Day Revenue Reconciliation — Compare revenue per episode to baseline. Identify undercoding patterns. HP leads final reinforcement training as needed.	Mar–Apr 2027	HP / Finance

SECTION 4 | EDUCATION MODULES — DETAILED PLAN

MODULE 1 — Office Visits (Antepartum & Postpartum) | First Draft: July 1, 2026 | Published: August 2026

- Target Audience: All OB/GYN physicians, NPs, PAs, front-desk, and HP billing staff.
- Format: Video lecture (**less than 15 minutes**) + downloadable quick-reference card.
- Content: What the global code was; why it's ending (validated by AMA June 2, 2026 webinar); E/M code selection rules (MDM vs. time); modifier TH usage; ICD-10-CM diagnosis coding; documentation requirements; common pitfalls and audit triggers.
- Super User Role: Review draft script for clinical accuracy. Pilot new coding on 10 real antepartum visit notes; compare to old global method and document findings.
- EHR Template Update: Coordinate with IT to activate updated antepartum/postpartum office visit note templates before pilot begins.
- Deliverable: Completed video + quick-ref card published to SharePoint module library.

MODULE 2 — Inpatient (Labor Management & Delivery) | Development: July–Aug 2026 | Published: August 2026

- Target Audience: OB hospitalists, laborists, delivery nurses (documentation awareness), HP billing staff.
- Format: Video lecture (**less than 15 minutes**) + inpatient coding scenario workbook.
- Content: New labor management codes — initial vs. subsequent day; straightforward vs. complex definitions; delivery-only codes (vaginal, vaginal + episiotomy, VBAC, primary C-section, repeat C-section); subsequent hospital care days; discharge management; new uterine tamponade code.
- Super User Role: Run tabletop scenarios — given a clinical summary, select the correct labor management + delivery code combination. Identify documentation gaps in current note templates.
- EHR Template Update: Revised inpatient delivery notes, labor management templates, and subsequent hospital care fields activated in EHR sandbox and reviewed by super users.
- Deliverable: Video + scenario workbook + updated inpatient note templates to SharePoint.

MODULE 3 — OB Comprehensive (Full Episode + Edge Cases) | Development: August 2026 | Published: Late August 2026

- Target Audience: All OB/GYN providers + HP billing leads (capstone — required for all).
- Format: Video lecture (**less than 15 minutes**) + live case-based Q&A; session + attestation quiz.
- Content: Complete pregnancy episode walkthrough; split care/transfer of care scenarios; VBAC and repeat C-section pathways; high-risk/MFM referral handoff coding; payer-specific nuances; patient communication about billing changes; auditing your own documentation.

- Super User Role: Co-present the live Q&A; session. Maintain a live FAQ document in SharePoint updated with questions arising during training.
- Deliverable: Video + quiz attestation system + FAQ document in SharePoint.

Super User Program

Super users are typically 2–4 experienced coders, billers, or clinically-trained staff who receive deep-dive training first, validate all education content, pilot new workflows, and serve as the first line of support for colleagues after go-live.

Super User Responsibility	Timeline
Complete advanced self-study on new CPT guidelines; review AMA June 2, 2026 webinar recording	June–July 2026
Review and validate Module 1 (Office Visits) video script and reference materials	July 2026
Pilot Module 1 coding on 10 real patient encounters; document findings	July 2026
Review updated antepartum/postpartum office visit EHR templates before provider rollout	July 2026
Review and validate Module 2 (Inpatient) content; run scenario-based testing	July–August 2026
Review updated inpatient delivery/labor management EHR templates	August 2026
Review and validate Module 3 (Comprehensive) content; build FAQ bank	August 2026
Co-present live Q&A; sessions during all-staff training	September 2026
Staff the SharePoint intake queue post-go-live; answer provider questions	Oct 2026–Mar 2027
Assist with 30/60/90-day post-go-live audit and coding accuracy reviews	Feb–Apr 2027

SECTION 5 | PHYSICIAN CODING INTAKE FORM & SHAREPOINT HUB

SCOPE OF THIS SYSTEM: This intake form and SharePoint hub are operated by the **OB Unbundling Consulting Team**. Our team provides clinical coding guidance and education to frontline OB/GYN physicians. We do **not** handle technology build, EHR configuration, payer contracting, or billing operations — those are managed separately by IT and HP. This system exists to answer one question: “As a physician, how do I correctly code this encounter under the new CPT 2027 rules?”

Physician Coding Consultation Intake Form

This form is completed by frontline OB/GYN physicians, NPs, and PAs when they have a question about how to code a specific encounter or clinical scenario under the new CPT 2027 framework. Submitted forms are routed directly to the consulting team and answered within the response window. Answers that apply broadly are added to the SharePoint FAQ.

SECTION A — ABOUT YOU

Q1. Your Full Name

Q2. Your Role / Title

☐ OB/GYN Physician (MD/DO) ☐ Maternal-Fetal Medicine (MFM) ☐ NP / PA ☐ Hospitalist OB ☐ Resident / Fellow ☐ Other

Q3. Your Practice Location / Site / Clinic Name

Q4. Best Email for Follow-Up Response

Q5. Have you reviewed the FAQ on SharePoint before submitting?

Checking the FAQ first helps us focus on new or complex questions.

☐ Yes — I looked and could not find an answer ☐ No — I wanted direct guidance from the team

SECTION B — TYPE OF QUESTION

Q6. What type of coding question do you have?

■ Which E/M code level to use for a specific visit ■ Whether and how to apply Modifier TH ■ Labor management — straightforward vs. complex ■ Which delivery code to use ■ Postpartum visit coding ■ What I need to document to support a specific code ■ Split care / shared care / transfer of care coding ■ High-risk or MFM referral handoff coding ■ Telehealth antepartum visit coding ■ Edge case or unusual clinical scenario ■ General education — I want to understand a concept ■ Other

(Select all that apply)

Q7. Which phase of care does your question relate to?

■ Antepartum (prenatal office visits) ■ Labor management (inpatient) ■ Delivery ■ Postpartum (inpatient) ■ Postpartum (office visit) ■ Multiple phases / full episode ■ Not sure

(Select all that apply)

Q8. Where did this encounter take place?

■ Office / Outpatient Clinic ■ Inpatient / Labor & Delivery Unit ■ Telehealth / Remote Visit ■ Hospital Outpatient Department ■ Not yet occurred — future guidance

SECTION C — CLINICAL SCENARIO

Do NOT include patient name, date of birth, MRN, or any other identifying information. Describe the encounter in clinical terms only.

Q9. Describe the clinical scenario

Include: gestational age, what occurred during the encounter, any complicating conditions, whether there was a prior cesarean, singleton vs. multiple gestation, any interventions performed.

Q10. What CPT code did you use — or were planning to use?

If you are unsure, write 'unsure' — that is fine. We will guide you.

Q11. What is your specific question?

Be as specific as possible. Example: 'I managed this patient's labor for 2 days before delivery — do I bill labor management on both days plus the delivery code on day 2?'

Q12. If documentation-related: What did you write in your note for this encounter?

Optional but helpful. Paste a brief summary of your note (de-identified). This helps us advise whether your documentation supports the code level.

SECTION D — URGENCY & RESPONSE

Q13. How urgent is your question?

■ Routine — Future guidance needed (respond within 5 business days) ■ Time-Sensitive — Recent encounter needs correction (respond within 2 business days) ■ Urgent — Patient is admitted now / encounter is today (respond within 24 hours)

Q14. May we use this question (de-identified) in the SharePoint FAQ for others?

Your name is never published. Only the question and answer are shared.

■ Yes — please share it to help the team ■ No — keep my question private

RESPONSE TIME COMMITMENTS

Routine (future guidance): Response within **5 business days**.

Time-Sensitive (recent encounter): Response within **2 business days**.

Urgent (patient admitted today): Response within **24 hours** — email the consulting team lead directly in addition to submitting this form.

How you will receive your answer: A consulting team member will reply directly to your email. If your question applies broadly, a de-identified version will be added to the SharePoint FAQ within one week.

SECTION 5B | SHAREPOINT HUB — FULL CONTENT SPECIFICATION

The OB Unbundling SharePoint hub is the team's single source of truth for all coding education, resources, and consultation tracking. It is maintained by the consulting team and is the primary destination physicians visit for self-service guidance. Below is the full content specification for each section — what lives there, who owns it, and how it is updated.

1. HOME / DASHBOARD PAGE

Owner: Consulting Team Lead

Updated: Real-time / as needed

- **Go-Live Countdown Widget:** Large visible countdown (days remaining) to January 1, 2027.
- **Module Completion Summary:** Progress bars showing % of OB/GYN physicians who have completed each of the 3 modules.
- **Open Consultation Queue:** Number of intake submissions currently awaiting response, so the team can monitor workload.
- **Latest Announcement Banner:** Pinned headline for the most recent AMA/ACOG update or team communication.
- **Quick Links Panel:** One-click buttons to: Submit a Coding Question (intake form), Browse the FAQ, View Education Modules, Contact the Consulting Team.
- **Consulting Team Contact Card:** Names, roles, email addresses of the coding consultants physicians can reach directly for urgent questions.
- **Important Dates Calendar:** Module deadlines, training session dates, September 1 training deadline, CMS RVU publication (Nov 2026), Go-Live (Jan 1, 2027).

2. SUBMIT A CODING QUESTION (INTAKE FORM)

Owner: Consulting Team

Updated: Always available

- **Intake Form Link — Prominently featured button** labeled 'Submit a Coding Question.' This is the primary call to action on the hub.
- **Form Instructions Summary:** Brief bullet list reminding physicians: (1) Do not include patient identifiers. (2) Be specific about the clinical scenario. (3) Check FAQ first. (4) Select your urgency level.
- **My Submissions Tracker:** A view where a physician can see their own past submissions, the current status (Submitted / In Review / Answered), and the response received.
- **Consulting Team Response Log:** Internal view (team only) showing all open items, assigned consultant, due date per SLA, and resolution notes.
- **Average Response Time Display:** Running metric showing current average response time (builds trust and accountability).

- **What We Answer vs. What We Don't:** Clear one-paragraph statement that this system is for clinical coding questions only — payer issues go to HP, EHR issues go to IT.

3. EDUCATION MODULES

Owner: Consulting Team / Education Team

Updated: Updated per module revision

- **Module 1 — Office Visits:** Embedded video (less than 15 min). Downloadable quick-reference card (PDF). Link to attestation quiz. Version number and last-updated date displayed.
- **Module 2 — Inpatient (Labor Management & Delivery):** Embedded video. Downloadable inpatient coding scenario workbook (PDF). Updated delivery note template. Attestation quiz link.
- **Module 3 — OB Comprehensive:** Embedded video. Full-episode case walkthrough document. Edge case guide (VBAC, transfer of care, split care, MFM handoff). Attestation quiz link.
- **Completion Tracker:** Table showing each physician's name and which modules are complete (green checkmark) vs. pending (yellow) vs. not started (gray). Updated automatically from quiz submissions.
- **Module Update Log:** Version history — date, what changed, and why — so physicians know if they need to re-review content.
- **Live Session Recordings:** Recordings of Q&A; sessions held with super users during training. Timestamped and searchable.
- **CE Credit Information (if applicable):** If modules are submitted for CME credit, include the CME tracking information here.

4. CODING RESOURCES LIBRARY

Owner: Consulting Team

Updated: Updated when AMA/ACOG guidance changes

- **2027 CPT Code Reference Sheet (PDF):** Side-by-side table — old code | deleted/revised | new code | new code description. One-page printable format.
- **Deleted Codes Alert List:** Clearly formatted list of all 17 deleted codes with a red 'DELETED' flag and the replacement approach for each. Easy to scan before submitting a claim.
- **Modifier TH Quick Guide:** When to apply it, when not to, how it appears on the claim, and which visits require it. Includes one-page cheat sheet PDF.
- **E/M Level Decision Grid (PDF):** Visual grid: rows = complexity level, columns = MDM elements required, time threshold. Physicians can cross-reference their encounter and select the right code.
- **ICD-10-CM Pregnancy Diagnosis Code Reference:** Organized by type — Z34 series (supervision of normal pregnancy by trimester), O chapter complication codes commonly used in OB, and how to pair diagnosis codes with new E/M codes.
- **Labor Management Decision Tree (PDF):** Flowchart — physician answers 6 yes/no questions about the labor. Chart outputs: 'Straightforward' or 'Complex' + correct code category. Printable for the L&D; unit.

- **Documentation Requirements Checklist (PDF):** Per-code checklist of what the physician's note must contain to support the code. One page per code category (antepartum E/M, labor management, delivery, postpartum E/M).
- **Split Care / Transfer of Care Guide:** Step-by-step guide for scenarios where the antepartum, labor, or delivery physician is different. How each provider codes their portion correctly.
- **Telehealth Antepartum Coding Guide:** How virtual prenatal visits are coded under the new framework, including the appropriate place of service code.
- **High-Risk / MFM Referral Coding Guide:** How the referring OB and the MFM consultant each code visits when care is shared or transferred.
- **Coding Scenario Library:** Growing collection of real (de-identified) scenarios with answers from the consulting team. Drawn from FAQ submissions. Searchable by topic.

5. FAQ / KNOWLEDGE BASE

Owner: Consulting Team (Super Users update weekly)

Updated: Weekly — every Friday by 5 PM

- **FAQ Organization:** Five tabbed sub-sections — (1) Antepartum, (2) Labor Management, (3) Delivery, (4) Postpartum, (5) General / Multi-phase.
- **Standard FAQ Entry Format:** Each entry includes: Question (de-identified as submitted), Answer (from consulting team), Related CPT Code(s), Related ICD-10 Code(s), Date Added, and whether a full Coding Scenario write-up is available.
- **Most Common Questions (Pinned):** Top 5 questions per phase are pinned at the top of each tab so the most frequent issues are immediately visible.
- **Search Functionality:** SharePoint search bar scoped to the FAQ library — physicians can type a keyword (e.g., 'VBAC' or 'augmentation') and see all relevant Q&A; entries.
- **'Was This Helpful?' Feedback:** Each FAQ entry has a thumbs up / thumbs down reaction so the team knows which answers need clarification.
- **Submit a New Question Button:** Embedded link to the intake form at the top of every FAQ tab — so if a physician can't find the answer, submitting is one click away.
- **FAQ Update Log:** Simple running list of new entries added each week, so regular users can quickly see what's new without scrolling everything.
- **Pre-Loaded FAQ at Launch (Minimum 20 Q&A;):** The consulting team and super users seed the FAQ before go-live using the most common questions identified during module pilots and training sessions.

6. ANNOUNCEMENTS & POLICY UPDATES

Owner: Consulting Team Lead

Updated: As updates are released by AMA, ACOG, or CMS

- **Pinned Critical Alert:** Any time the AMA, ACOG, or CMS issues a material change to 2027 coding guidance before go-live, a pinned alert with red banner is posted here immediately.

- **CMS RVU Publication (November 2026):** When CMS publishes final relative values in November 2026, the consulting team posts a plain-language summary of any reimbursement impacts relevant to physician documentation.
- **Training Schedule & Registration Links:** Calendar of all live training sessions, Q&A; events, and refresher sessions. Includes video conference links and add-to-calendar functionality.
- **Session Recordings Archive:** All recorded training sessions organized by date and topic. Physicians who missed a session can watch on demand.
- **AMA / ACOG Resource Links:** Curated, maintained list of official external links — AMA coding primer, ACOG payment page, SMFM guidance, AAPC OB coding resources — verified as current.
- **Policy Version Control:** Whenever a coding policy document is updated, the old version is archived (not deleted) and the new version is labeled with the effective date. Physicians can always see what changed and when.
- **Go-Live Countdown & Status:** Running announcement of go-live readiness — as milestones are hit (all modules published, all providers trained, etc.), the team posts a status update confirming completion.

SECTION 6 | CODING EXAMPLES — HOW OB PHYSICIANS WILL CODE IN 2027

The following five examples illustrate how an OB/GYN physician will code under the new CPT 2027 framework. **Labor management codes 59080–59083 are confirmed by the AMA.** Delivery codes (vaginal, VBAC, cesarean) retain placeholder designations pending the full CPT 2027 book publication; final numbers will be confirmed in November 2026 when CMS publishes RVU values. All E/M code levels and ICD-10 codes are current and final.

EXAMPLE 1 — OUTPATIENT — Routine Antepartum Visit (Office)

Setting: Outpatient Office Visit — Established Patient

Clinical Scenario: A 28-year-old patient at 20 weeks gestation presents for a routine prenatal visit. The physician reviews labs, checks fundal height, documents fetal heart tones, reviews blood pressure and weight gain. Patient has minor round ligament pain questions. No significant comorbidities. Total encounter time: 22 minutes.

Phase / Service	CPT Code	Modifier	Notes
Antepartum E/M Visit	99213	TH	Established patient, low complexity MDM; 20–29 min encounter. Modifier TH appended to identify as obstetric/prenatal service.

ICD-10-CM Diagnosis: Z34.20 — Encounter for supervision of normal pregnancy, second trimester

Documentation Tips: Document: date, GA, BP, weight, fundal height, FHT, labs reviewed, patient concerns addressed. Select 99213 (low complexity MDM) or use time if ≥20 minutes total. Modifier TH is appended by billing staff at charge submission.

EXAMPLE 2 — OUTPATIENT — High-Risk Antepartum Visit (Complex Medical Problem)

Setting: Outpatient Office Visit — Established Patient, High-Risk

Clinical Scenario: A 32-year-old patient at 28 weeks with gestational diabetes requiring insulin. The physician reviews glucose logs, adjusts insulin dosing, orders additional fetal monitoring (BPP/NST), counsels on diet and hypoglycemia risk, and coordinates with MFM. Multiple data points reviewed and management changed. Total time: 38 minutes.

Phase / Service	CPT Code	Modifier	Notes
Antepartum E/M Visit — Moderate Complexity	99214	TH	Established patient, moderate complexity MDM. New/worsening chronic condition requiring additional work-up or management change. 30–39 min.

ICD-10-CM Diagnosis: O24.414 — GDM in pregnancy, insulin-controlled; Z34.23 — Supervision of normal pregnancy, third trimester

Documentation Tips: Document: glucose log review, insulin adjustment rationale, additional testing ordered, MFM coordination note. MDM must reflect NEW management decision (insulin dose change) or complexity of data reviewed. Time documentation is an alternative — capture total face-to-face time in the note.

EXAMPLE 3 — INPATIENT — Straightforward Labor Management + Same-Day Vaginal Delivery

Setting: Inpatient Hospital — Labor & Delivery Unit

Clinical Scenario: A 30-year-old G2P1 patient at 39+2 weeks presents in active labor. Singleton vertex presentation. Routine maternal/fetal monitoring; no physician intervention needed. Normal progression of labor. No prior cesarean. No complicating medical conditions. She delivers vaginally the same day with a first-degree laceration (repaired at delivery). Physician manages the full labor and delivers the baby.

Phase / Service	CPT Code	Modifier	Notes
Labor Management — Initial Day, Straightforward	59080		First calendar date; ALL straightforward criteria met: singleton vertex, routine monitoring, normal fetal monitoring, normal labor progression, stable conditions, no prior C-section.
Vaginal Delivery	59XX5		Stand-alone delivery code. Includes delivery of fetus & placenta, episiotomy if performed, and repair of 1st/2nd-degree laceration. Both 59080 and 59XX5 may be reported same day.

ICD-10-CM Diagnosis: O80 — Encounter for full-term uncomplicated delivery

Documentation Tips: Document labor management note separately from delivery note. Labor management note must support ALL straightforward criteria. Delivery note must document fetal presentation, laceration degree, repair performed, and placenta delivery. Note: Routine same-day postpartum care is INCLUDED in the delivery code — do not bill separately.

EXAMPLE 4 — INPATIENT — Normal Vaginal Delivery with Multi-Day Labor (Induction)

Setting: Inpatient Hospital — Labor & Delivery / Postpartum Unit

Clinical Scenario: A 26-year-old G1P0 at 40+1 weeks admitted on Day 1 for elective induction. Singleton vertex. Induction begins with Pitocin augmentation; routine progression. No complications on Day 1. Labor continues into Day 2 — she delivers vaginally on Day 2. Postpartum inpatient stay continues through Day 4 (discharge).

Phase / Service	CPT Code	Modifier	Notes
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Day 1 — Labor Mgmt, Initial Day, Straightforward	59080		Routine induction/augmentation qualifies as straightforward. First calendar day of admission.
Day 2 — Labor Mgmt, Subsequent Day, Straightforward	59082		Second calendar day; continued labor management, still meets straightforward criteria.
Day 2 — Vaginal Delivery (same day)	59XX5		Delivery reported on Day 2 same date as subsequent labor management — both billable.
Day 3 — Subsequent Hospital Care	99232		Post-delivery inpatient management day; use appropriate subsequent hospital care E/M.
Day 4 — Discharge Management	99238		Discharge day; use discharge management code. 99238 (≤30 min) or 99239 (>30 min).
ICD-10-CM Diagnosis: O80 — Uncomplicated delivery; Z39.0 — Encounter for care immediately after delivery (post-delivery days) Documentation Tips: Billing generates 5 separate claims for this 4-day episode — a fundamental change from 1 global claim. Document each day's labor management note. Subsequent hospital care notes must support chosen E/M level. HP should verify payer has activated new code rates before claims are submitted.			

EXAMPLE 5 — INPATIENT — Primary Cesarean Delivery (Complex Labor + C-Section)

Setting: Inpatient Hospital — Labor & Delivery / Postpartum Unit

Clinical Scenario: A 29-year-old G1P0 at 39+4 weeks admitted in labor. Singleton vertex, no prior cesarean. Physician initiates labor management; after 20+ hours, labor arrest (active phase arrest) is diagnosed. Decision is made for primary cesarean. Physician performs the C-section. Patient remains inpatient through Day 4, then discharged.

Phase / Service	CPT Code	Modifier	Notes
Day 1 — Labor Mgmt, Initial Day, Straightforward	59080		At start of labor, all criteria met — straightforward. Report based on the day's status.
Day 2 — Labor Mgmt, Subsequent Day, COMPLEX	59083		Labor arrest = deviation from normal progression → elevates to COMPLEX. Any deviation from the 6 straightforward criteria triggers complex.
Day 2 — Primary Cesarean Delivery	59XX7		Stand-alone primary C-section code. Includes abdominal/uterine incision, fetal/placenta delivery, and closure. Billable same day as labor management.

Day 3 — Subsequent Hospital Care	99232		Post-C-section inpatient management. Document wound check, pain management, ambulation.
Day 4 — Discharge Management	99238		Discharge day management code. Provide discharge instructions, prescriptions, follow-up plan.
ICD-10-CM Diagnosis: O63.1 — Prolonged second stage of labor; O82 — Encounter for cesarean delivery without indication Documentation Tips: Key point: The SAME labor can shift from straightforward to complex mid-course — document the change clearly. The complex upgrade is triggered by ONE deviation from the 6 criteria. Complex labor management code (59083) may generate higher reimbursement than straightforward (59080/59082). Coordinate with HP to confirm complex labor management rates are covered under renegotiated contract.			

SECTION 7 | NEW BILLING WORKFLOW

The table below maps the current billing action to the new required action under CPT 2027. Use this to update EHR charge capture templates and train HP billing staff.

Service	Current (2026 & Prior)	New (Jan 1, 2027)
First prenatal visit	Count toward global 59400/59510	E/M 99202–99215 + modifier TH
Subsequent antepartum visits	Count toward global package	E/M 99211–99215 per visit + modifier TH
Antepartum-only (≤3 visits)	59425	DELETED — E/M + TH each visit
Antepartum-only (4–6 visits)	59426	DELETED — E/M + TH each visit
Labor mgmt — initial day, straightforward	Included in global code	New Code 59080 — initial, straightforward
Labor mgmt — subsequent day, straightforward	Included in global code	New Code 59082 — subsequent, straightforward
Labor mgmt — initial day, complex	Included in global code	New Code 59081 — initial, complex
Labor mgmt — subsequent day, complex	Included in global code	New Code 59083 — subsequent, complex
Vaginal delivery	59400 (global) or 59409 (delivery only)	New Code 59XX5 — vaginal delivery
Vaginal delivery + episiotomy	Variant of 59400/59409	New Code 59XX5 — incl. 1st/2nd-degree repair
VBAC	59610 / 59612	New Code 59XX6 — VBAC delivery
Cesarean — primary	59510 (global) or 59514 (delivery only)	New Code 59XX7 — primary cesarean
Cesarean — repeat	59618 / 59620	New Code 59XX8 — repeat cesarean
3rd-degree laceration repair	No dedicated code	New Code 59X11 — 3rd-degree repair

4th-degree laceration repair	No dedicated code	New Code 59X12 — 4th-degree repair
Postpartum hospital days	Included in global code	Subsequent hospital care codes per day
Hospital discharge	Included in global code	Discharge management code (99238/99239)
Postpartum office visit(s)	59430 — DELETED	E/M 99211–99215 per visit + modifier TH
Uterine tamponade	No dedicated code	New dedicated procedure code
Obstetrical ultrasound / fetal MRI	Always billed separately	No change — billed separately

Labor Management — Straightforward vs. Complex Criteria

ALL six criteria must be met for straightforward. One deviation triggers complex.

Straightforward Criteria (ALL must be met)	Complex Triggers (ANY one elevates to complex)
Singleton vertex presentation	Multiple gestation (twins, triplets, etc.)
Routine maternal/fetal monitoring	Non-vertex presentation (breech, transverse)
Fetal monitoring not requiring physician intervention	Fetal monitoring requiring physician intervention
Normal progression OR routine induction/augmentation	Abnormal labor progression / labor arrest
Stable medical conditions — no additional management	Deteriorating or unstable medical condition during labor
No prior cesarean delivery	Prior cesarean (VBAC attempt = complex by default)

AMA-Confirmed Rules: Same-Day Complexity Upgrade & Planned Cesarean

RULE 1 — Same-Day Complexity Upgrade	AMA Confirmed
Scenario: Labor management begins as straightforward (59080 or 59082) but during the same calendar date transitions to complex labor management.	
Correct Billing: Report ONLY the complex labor management code (59081 or 59083) for that calendar date. Do not report both straightforward and complex codes for the same day.	Yes — explicitly documented in AMA CPT 2027 maternity care coding guidance.
RULE 2 — Planned / Scheduled Cesarean	AMA Confirmed
Scenario: A patient is scheduled for a planned or elective cesarean delivery with no trial of labor.	

Correct Billing: Do NOT report any labor management code (59080–59083). Report only the cesarean delivery code. Labor management codes apply only when labor management services are actually provided.	Yes — explicitly documented in AMA CPT 2027 maternity care coding guidance.
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Both rules above are sourced directly from AMA CPT 2027 maternity care guidance and confirmed via the AMA June 2, 2026 coding primer webinar. Compliance with these rules is essential to avoid claim denial or audit exposure.

SECTION 8 | HP — PAYER CONTRACTING & EHR READINESS

HP Payer Contracting Action Items

ACOG recommends all health plans transition no later than **September 1, 2026**. HP must proactively lead all payer outreach to ensure new code rates are in place before claims are submitted. HP leadership should be engaged early to coordinate contract strategy across all contracted payers.

- HP generates a full list of all contracted payers currently reimbursing global OB codes.
- HP coordinates with HP leadership to develop payer outreach and negotiation strategy.
- HP contacts each payer's provider relations representative to confirm their 2027 transition plan.
- HP negotiates per-code rates for: antepartum E/M visits, labor management codes (straightforward/complex, initial/subsequent), delivery codes, and postpartum E/M visits.
- HP confirms payers will accept modifier TH without requiring prior authorization for antepartum visits.
- HP requests written confirmation of new fee schedules before October 31, 2026.
- HP flags payers that intend to maintain global payment structures (requires escalation to HP leadership).
- HP updates all contracts in credentialing/contracting management system.
- HP coordinates separate outreach to Medicaid managed care plans (MCOs) which may have different timelines.
- HP publishes payer-by-payer contract status to SharePoint HP Payer Information section.

EHR / EMR Update Checklist

Area	Action Required
Charge Capture	Remove all deleted global OB codes (59400, 59510, 59425, 59426, 59430, etc.) from order sets and charge capture screens.
New Code Entry	Add all new labor management codes, revised delivery codes, new laceration repair codes, modifier TH to charge master. Coordinate with HP on fee schedule mapping.
Office Visit Templates	Activate updated antepartum and postpartum office visit note templates with MDM elements and time capture. (Complete July 2026 — before Module 1 pilot.)
Inpatient Templates	Activate updated labor management documentation template, delivery note template, and subsequent hospital care templates. (Complete August 2026 — before Module 2 pilot.)
Reporting	Build utilization report to monitor new code usage vs. benchmarks. Flag if deleted codes appear on any 2027 claim.
Sandbox Testing	Test all new codes and templates in sandbox environment before November 30, 2026.
Go-Live Validation	First-week 2027 claims review: confirm all claims use new codes; zero tolerance for deleted codes.

SECTION 9 | ROLES & RESPONSIBILITIES

Department Medical Director

- Executive sponsor. Ensures physician buy-in and mandates training completion by September 1.
- Approves final education module content.
- Signs off on go-live readiness checklist.
- Communicates urgency and importance to all OB/GYN physicians.

Practice Manager / Administrator

- Owns overall project timeline and accountability tracker.
- Coordinates with HP on payer contracting outreach strategy.
- Manages SharePoint hub setup and maintenance.
- Tracks training completion rates and escalates non-completion.

HP (Health Plan / Billing Operations)

- Leads all billing staff training on new claim submission workflow.
- Coordinates with HP leadership on payer contract renegotiations.
- Manages payer outreach, per-code rate negotiations, and fee schedule confirmations.
- Runs needs assessment and all post-go-live claims audits (30/60/90 day).
- Monitors claim acceptance/denial rates beginning January 2027.
- Maintains HP Payer Information section of SharePoint hub.
- Staffs intake queue for all HP/billing/payer-related questions.

Super Users (2–4 Staff)

- First to learn new coding guidelines — serve as internal subject matter experts.
- Review and validate all three education modules.
- Pilot new coding workflows before full deployment.
- Review updated EHR templates before provider rollout.
- Staff the SharePoint FAQ/intake queue post-go-live.
- Lead live Q&A; sessions during all-staff provider training.

IT / EHR Team

- Build and activate updated E/M documentation templates (office visit July, inpatient August).
- Remove deleted codes from charge capture; add new codes to charge master.
- Set up SharePoint hub and intake form (Microsoft Forms integration).
- Run sandbox testing of all new code sets before go-live.
- Build and maintain utilization monitoring reports.

Education Team

- Produce all three video modules on schedule (each less than 15 minutes).
- Coordinate with super users for content review at each draft stage.
- Publish modules to SharePoint; manage attestation quiz system.
- Update content promptly if AMA/ACOG/CMS guidance is revised before go-live.

All OB/GYN Physicians, NPs, PAs

- Complete all three training modules by September 1, 2026.
- Complete attestation quiz for each module.
- Update documentation habits to support E/M level selection and time capture.
- Use SharePoint intake system for any coding questions after training.
- Participate in post-go-live support sessions and audits as needed.

SECTION 10 | MASTER READINESS CHECKLIST

All items must be completed before advancing to the next phase. HP is responsible for all payer/billing checklist items.

JUNE 2026 — PROJECT LAUNCH

- Core project team assembled and roles assigned (including HP lead)
- AMA June 2, 2026 webinar recording distributed to core team
- AMA 2027 CPT maternity code documentation obtained and shared
- HP needs assessment completed (current code usage, volume, payer list)
- Super users identified and enrolled in advanced training
- SharePoint hub structure designed and build started
- Microsoft Forms intake form drafted and routing logic configured

JULY 2026 — MODULE 1 & TEMPLATE UPDATE

- Module 1 (Office Visits) video — first draft completed by July 1
- Super users reviewed Module 1 draft and provided feedback
- Updated antepartum/postpartum office visit EHR templates activated (sandbox)
- Super users reviewed updated office visit templates for accuracy
- Pilot coding exercise completed on 10 antepartum encounters
- Module 1 quick-reference card designed

AUGUST 2026 — MODULES 2 & 3 + INPATIENT TEMPLATES

- Module 1 finalized and published to SharePoint
- Module 2 (Inpatient) video completed and super user reviewed
- Updated inpatient labor management and delivery EHR templates activated (sandbox)
- Super users reviewed and approved inpatient EHR templates
- Inpatient coding scenario workbook published

- Module 3 (Comprehensive) video completed and super user reviewed
- Attestation quiz built and tested for all three modules
- FAQ bank seeded with top 20 anticipated questions
- All three modules live in SharePoint by late August

SEPTEMBER 2026 — PROVIDER TRAINING

- All OB/GYN physicians scheduled and trained — all three modules by September 1
- Attestation records captured for all providers
- HP billing staff training completed (new workflow, modifier TH, prior auth)
- Front desk patient communication scripts distributed
- Intake system (Microsoft Form → SharePoint) live and tested
- Super users confirmed as go-to contacts in SharePoint FAQ

OCTOBER–NOVEMBER 2026 — HP TECHNICAL READINESS

- HP: All major payer contracts renegotiated with new per-code rates
- HP: Written fee schedule confirmations received from all payers
- HP: Medicaid managed care plans confirmed on new code structure
- HP: Payer status tracker fully updated in SharePoint
- IT: EHR sandbox testing of all new codes completed
- IT: Old global codes removed from live charge capture
- IT: New labor management and delivery codes active in live system
- IT: Utilization monitoring report built and tested
- Dept. Director: Go-live readiness sign-off obtained

JANUARY 1, 2027 — GO-LIVE

- Zero claims submitted with deleted global codes

- All antepartum/postpartum visits billed with E/M + modifier TH
- Labor management codes applied correctly (initial/subsequent, straightforward/complex)
- Delivery codes applied as stand-alone (not bundled with labor management)
- SharePoint hub active; HP intake queue monitored daily
- Super users available for first-week live support

FEBRUARY–APRIL 2027 — POST-GO-LIVE

- HP: 30-day denial rate review completed
- HP: Documentation quality audit completed
- HP: 60-day revenue per pregnancy comparison to baseline
- HP: 90-day revenue reconciliation and undercoding correction plan issued
- Final provider training reinforcement sessions held if needed
- Playbook updated with lessons learned

SECTION 11 | REFERENCES & KEY RESOURCES

#	Source	Title	URL
1	American Medical Association	CPT 2027 Maternity Care Services Code Changes (Official AMA Page)	ama-assn.org/practice-management/cpt/cpt-2027-maternity-care-services-code-changes
2	American Medical Association	FAQs: CPT 2027 Maternity Care Services Code Changes	ama-assn.org/practice-management/cpt/faqs-cpt-2027-maternity-care-services-code-changes
3	American Medical Association	CODING PRIMER WEBINAR — June 2, 2026: 'A Coding Primer: Previewing the CPT 2027 Restructure for Maternity Care Services Codes'	ama-assn.org/membership/events/coding-primer-previewing-cpt-2027-restructure-maternity-care-services-codes
4	American Medical Association	Health Plan Primer — CPT 2027 Restructure for Maternity Care Services (Payer-Facing)	ama-assn.org/membership/events/health-plan-primer-previewing-cpt-2027-restructure-maternity-care-services
5	American Medical Association	CPT Maternity Care Services Codes and Guidelines (PDF)	ama-assn.org/system/files/cpt-maternity-care-codes-guidelines.pdf
6	ACOG	After Years of ACOG Advocacy, AMA Releases New Obstetric Codes (April 2026)	acog.org/news/news-releases/2026/04/ama-releases-new-obstetric-codes
7	ACOG	Payment for Obstetric Services — Coding Library	acog.org/practice-management/coding/coding-library/payment-for-obstetric-services
8	ACOG	Payment Advocacy for Obstetric Services	acog.org/advocacy/policy-priorities/payment-parity-for-obstetric-services
9	Society for Maternal-Fetal Medicine (SMFM)	2026 Code Changes and Beyond	smfm.org/news/2026-code-changesand-beyond
10	Healthcare Dive	AMA Creates New Maternity Care Coding System	healthcaredive.com/news/ama-maternity-code-overhaul-pregnancy-obgyn-cpt/813363/
11	Healio	New Obstetric Codes Will Replace Bundled Payment — Here's What to Know (May 2026)	healio.com/news/womens-health-ob-gyn/20260520/new-obstetric-codes-will-replace-bundled-payment
12	Contemporary OB/GYN	AMA, ACOG Announce Restructuring of Maternity Care Coding for 2027	contemporaryobgyn.net/view/ama-acog-announce-restructuring-of-maternity-care-coding-for-2027

1 3	California Medical Association	AMA Announces Major Overhaul of Maternity Care CPT Codes Beginning in 2027	cmadocs.org/newsroom/news/view/ArticleId/51210
1 4	Kovor RCM	Navigating the 2027 CPT Maternity Care Overhaul: Implications for OB/GYN and MFM Revenue Cycle	kovorcm.com/2027-cpt-maternity-care-coding-overhaul/
1 5	Billing Freedom	End of Global OB CPT Codes in 2027 — New AMA Maternity Billing Rules Explained	billingfreedom.com/end-of-global-ob-cpt-codes-in-2027-a-ma-maternity-billing-rules/

This playbook was prepared in June 2026 based on AMA, ACOG, SMFM, and industry guidance available at time of writing, including the AMA Coding Primer Webinar of June 2, 2026. CPT 2027 codes are finalized; final CMS RVU values will be published November 2026. Payer-specific policies may evolve — verify current guidance at ama-assn.org and acog.org prior to implementation. This document does not constitute legal or compliance advice.